

REPORT

THE ROLE OF SCHOOLS OF PHARMACY AND THE PHARMACISTS IN THE RATIONAL USE OF PSYCHOACTIVE DRUGS

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INTRODUCTION

1. Psychoactive substances have three categories, one is illicit substances, e.g. heroine, cocaine, cannabis, etc. The second is tobacco and alcohol which are legal in most societies. The third is a very important group which consist of narcotic and psychotropic drugs legally available in countries with a prescription by a physician. These are opiates, tranquillisers, hypnotics, anti-depressants, etc. which are the focus of concern regarding their excessive use. The medical profession provides a pharmacological response to non-medical problems. The responsible role of physicians and pharmacists is a key to the rational use of these drugs.
2. At present three international drug control treaties exist to assist countries and the UN control agencies and especially WHO, to protect population from the harm psychoactive substances are likely to produce and to allow their therapeutic properties to be exploited. The first treaty is the 1961 Single Convention on Narcotic Drugs which is mainly concerned with control of opiates including morphine, codeine and heroine. It also controls coca plant and cocaine and the cannabis plant. The second treaty is the 1971 Convention on Psychotropic substances which controls a wide range of substances, among them are hallucinogens, stimulants, depressants and tranquillisers, etc. The third treaty is the 1998 convention on Psychotropic and Narcotic Drugs and illicit trafficking which controls the precursors of narcotic and psychotropic substances but is primarily intended to take away the financial profits gained by the traffickers.
3. WHO has been given responsibility by the first two treaties to evaluate the benefit and risk ratio of psychoactive substances and to recommend their control to the United Nations. A psychotropic and narcotic drug unit (PND) undertakes this responsibility as its primary function in collaboration with experts, member states, and UN agencies like INCB, UNDCP and the UN Commission on Narcotic Drugs. A second responsibility of this unit is to help in the education of the health care professionals who are responsible for providing these substances and to monitor their effects on the population.

ACTIVITIES BY WHO

4. WHO has carried out numerous activities to acquaint leaders in health fields around the world with danger associated with the use of psychoactive drugs with dependence liability. The major responsibility of the PND Unit is to recommend substances based on their benefit risk ratio to the Secretary General of the United Nation for International control. These recommendations are debated at the UN Commission on narcotic drugs in Vienna. The Executive Board of WHO has recognized that in addition to these activities the members of the medical profession have to prescribe these drugs rationally. This requires training for physicians and other health care professionals like pharmacists and nurses who are partners with physicians to select drugs, administer them to the patients and monitor their use.
5. A WHO book, "Psychoactive Drugs, improving prescribing practices" is the result of discussions in Moscow regarding the training of health care professionals to promote the rationale use of these drugs. It provides a wide ranging analysis of what can be done to reduce the misuse of these drugs without compromising appreciation for their therapeutic value. In another meeting convened in London, reviewed "the role of medical schools in promoting the rationale use of psychoactive drugs". It recommended that better documentation be obtained world wide on adverse consequences of inappropriate prescriptions and that better educational efforts are needed. In another international meeting convened in London on "the role of schools of pharmacy in the rationale use of psychoactive drugs" in 1987. The conclusions were that pharmacists could provide very substantial assistance towards the rationale use of these drugs and the control of illicit drugs. But this will depend on two conditions, the first that pharmacists must be given opportunities to use their knowledge and skills by the medical and other health care professionals, by the authorities and regulatory bodies including law enforcement agencies. The second is that some adjustments need to be made to the education and training of pharmacists in relation to these drugs. The UN Commission on Narcotic Drugs in February 1988 passed a resolution entitled "the role of pharmacists in the prevention of the abuse of narcotic drugs and psychotropic substances", thus strengthening the role of pharmacists further. This resolution was distributed to all UN members by the Secretary General of the United Nations.

INFORMATION ON CHINA

6. WHO has also convened a national meeting for the Chinese representing all schools of pharmacy and international experts in Chengdu in 1988. They discussed this subject within the Chinese context. The salient points were that a speciality needed to be developed in China promoting the rationale use of drugs and medicines. The pharmacists should take their responsibility in this regard. Activities must be supported through education at student level and continuing education level. Regular national seminars are needed. A network must be established for drug information and encourage drug utilization studies. Data thus generated will met the need of professional societies of

pharmacists and physicians. Exchange and interchange of data between school of pharmacies in China is important. Meetings need to be held at national and local level, covering teaching and curriculum development in the field of clinical pharmacy. This will also include participants from the nursing and medical professions. The primary education of pharmacists should include material in medicine and medical clinical practice, in addition to the pharmaceutical subjects. Pharmacy teachers should join hospital committees for procuring, dispensing and monitoring the use of drugs to the patients. The national pharmaceutical teaching programme in China be modified to include the rationale use of drugs and prevention of drugs abuse courses, regulation control of drugs and education regarding medical subjects. Text books and pharmaceutical journals must be modified and include subjects mentioned above.

INFORMATION OF PAKISTAN

7. In 1990 I worked for a month with Dr. S.I. Ahmad, Faculty of Pharmacy, Karachi University in Pakistan. I visited six out of all 7 schools of pharmacy and reviewed pharmacy education with special reference to the great problem of drug abuse in Pakistan. Drug dependence effects about three million people out of 120 million, most of these are users of heroine, cannabis, as well as licit psychotropic drugs. The Pakistan Narcotic Control Board has responsibility for reducing both the availability of the illegal substances as well as in reducing demand for illicit drugs. In 1967, the Pharmacy Act was passed and amended six years later. This governs the training of pharmacists and pharmacy technicians. Degree courses were offered by the 7 schools of pharmacy all in public sector but their number has gone up to 12 now including those in the private sector. Graduates in pharmacy were generally employed by the pharmaceutical industry and training programmes in the universities have remained largely chemistry oriented. There was virtually no content of pathology or clinical medicine. The syllabi remained as they were 40 years ago. Some training was imparted in drug control but no one graduate in the 7 schools surveyed was aware of the three international drug control treaties to which Pakistan is a signatory. Nowhere in Pakistan, not even in the colleges of medicine, a training in cost effectiveness of pharmacotherapy was imparted. Collaboration between schools of pharmacy was restricted to the exchange of examiners. There was no official forum where teachers need to meet and solve their professional problems.
8. I had another opportunity to work with the Ministry of Health, WHO and UNDP in 1995/96 and I tried to promote the rational use of drugs. I reviewed all medicinal drugs, including psychoactive drugs in Pakistan. I suggested measures for improving the present situation. Particular attention was paid to the selection, availability and rationale use of essential drugs. Reference was also made to procurement policy, quality control, drug registration and control and the education of doctors and pharmacists. I also studied the dissemination of drug information, prescribing practices and the availability of prescription drugs without prescription. The University Grant Commission (UGC) has the final control of pharmacy education. I was glad that a positive decision of this

Commission recently taken to introduce in the syllabi of bachelor of pharmacy courses clinical and medical subjects. While in the Punjab province there is one pharmacist per 200 beds in teaching hospitals and district headquarters hospitals, this level of provision is far from being achieved elsewhere in the province. Further efforts are needed to improve the training of pharmacists in relations to adverse drug reactions, drug utilization studies, prescription surveys and other areas associated with clinical medicine. Practice of this kind would allow pharmacists to cooperate with doctors and help to reduce irrational prescribing. Pharmacists can be expected to become involved in preventive education, especially related to drug abuse and irrational prescribing. Doctors undoubtedly need advice on drug therapy to counteract the frequently aggressive approaches of the pharmaceutical industry. The new drug policy of Pakistan of 1997 aims at improving the training and responsibility of pharmacists further is welcome.

9. REFERENCES

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